

Psychosocial Assessment

CMT: CMT-0005 (SR)

MCG Health

Shared Resources
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- Introduction
- Psychosocial Assessment
- Interventions
 - Psychosocial Interventions
 - Interventions for Caregivers
- References
- Footnotes

Introduction

The psychosocial assessment is the evaluation of the social and psychological environment, home, and community resources of the patient, family, and caregiver. This assessment is usually performed by the care manager or home healthcare agency clinician. The purpose is to review actual or potential factors that may affect the patient's recovery when planning follow-up care, rehabilitation, or continued care. The assessment should be performed at the time of service initiation and before discontinuation of services to ensure patient safety. The assessment should be tailored to the specific circumstances involved, regardless of social status, educational level, or race and ethnicity. These circumstances can be diagnosis-related or age-related.(1)(2) After the assessment is performed, a plan should be developed to address the documented needs.(3)

The assessment is designed for use by a generalist. It is divided into broad categories of factors that may impact the patient and caregiver. Not every factor may be relevant for every patient; therefore, not every element in the assessment needs to be addressed. Once issues are identified from the assessment, a referral to home care, specialist (eg, counselor, psychosocial nurse, social worker), or community resource may be needed.

Long-term caregiving is a chronic stressor and may produce psychological and physiologic changes. Caregivers benefit from the same type of psychosocial interventions as patients. They require concrete information about their caregiving tasks and the patient's condition.(4)(5)

Psychosocial Assessment

- **Goals** of assessment include identification of(6)(7):
 - Feelings about diagnosis and beliefs about condition
 - Patient or caregiver motivation and readiness to follow plan of care
 - Psychiatric comorbidity (eg, depression, substance-related disorder)
 - Coping mechanisms
 - Caregiver availability and support system
 - Impact on patient's or caregiver's lifestyle, beliefs, and family and social roles
 - Ability of patient to return to usual activities (eg, hobbies, work, sports)
 - Factors that may impede adherence to treatment, and assistance in identifying patient and caregiver strengths and resources to manage treatment(2)
- **Psychosocial History**(1)(8)
 - Current living status
 - Married
 - Lives alone
 - Support system (eg, lives with caregiver)
 - Animals or pets in domicile (eg, apartment, house)
 - Housing status
 - Homeless
 - Shelter
 - Temporary living arrangements (eg, lives with friend)
 - Domicile (eg, apartment, house)
 - Available utilities(9)
 - Heating or cooling present
 - Electrical availability (eg, for lights, appliances)
 - Water

- Shares immediate living quarters (eg, bed, bedroom, bathroom, domestic areas)
- Stairs in living location
- Safety in current living location
- Recent relocation
- Other
- Educational level
 - Some schooling
 - High-school graduate
 - Trade school
 - Some college
 - College graduate
 - Advanced degrees
- Cultural background⁽¹⁰⁾
 - Citizenship
 - Ethnic heritage
 - Primary language
- Employment status
 - Unemployed
 - Employed
 - Current plan to return to work
 - Retired
 - Not applicable
- Psychiatric disorder or diagnosis (present or historical)
 - Depression or anxiety
 - Personality disorder (eg, borderline, antisocial)
 - Psychological disorder (eg, schizophrenia)
 - Hospitalization or diagnosis with psychiatric or behavioral problem
 - Presence or history of physical, emotional, or sexual abuse⁽¹¹⁾
 - Other
- Alcohol or drug use disorder (present or historical)⁽¹⁾
 - Alcohol use
 - History of alcohol use disorder
 - Alcohol use currently
 - None
 - Rare (eg, 1 drink a week or less)
 - Occasional (eg, 2 to 7 drinks a week)
 - Frequent (eg, 7 or more drinks a week)
 - Drug use
 - History of drug use disorder, type of drug:
 - Stimulants (eg, cocaine (crack), amphetamines, MDMA (3,4-methylenedioxy-methamphetamine) or ecstasy)
 - Depressants or sedatives (eg, barbiturates, benzodiazepines, methaqualone)
 - Cannabis (ie, marijuana)
 - Hallucinogens (eg, LSD (lysergic acid diethylamide), PCP (phencyclidine), mescaline)
 - Narcotics (eg, opium, morphine, codeine)
 - Drug use currently
 - None
 - Rare
 - Occasional
 - Frequent
 - Frequent episodes causing urge to use
- Presence of life changes in last 6 months
 - Change in marital status
 - Moving
 - Job loss
 - Birth
 - Death of someone close
 - Significant illness or injury
- Stress level perception
 - Low
 - Moderate
 - High
- Current health perception⁽¹²⁾

- Patient rating of health status
 - Terminal
 - Poor
 - Temporary health issues
 - Good
 - Excellent
- Pain rating. See Pain Management  SR.
 - None
 - Low
 - Moderate
 - High
 - Extreme
- Pain medication type
 - Over-the-counter
 - Opioids
 - Other
 - Not applicable
- Pain medication adequacy
 - Sufficient
 - Inadequate
 - Breakthrough
 - Not applicable
- Health literacy, patient able to(10)(13):
 - Understand directions on appointment slips
 - Understand medical forms
 - Express confidence in:
 - Filling out medical forms
 - Following directions on medication bottle
 - Ability to understand verbal or written information from healthcare provider about medical condition
 - Read alone or requires another to read hospital materials
- Presence of advance directives (eg, living will, power of attorney for medical decisions)
- **Coping and Cognitive Abilities of Patient and Caregivers(14)**
 - Psychological and emotional ability to function independently
 - Patient is able to self-manage.
 - Patient requires help of others for self-management.
 - Caregiver is able to manage patient independently, if needed.
 - Caregiver is able to manage patient with outside support, if needed.
 - Neither patient nor caregiver can manage patient.
 - Previous level of highest functioning
 - Has managed difficult situations in past
 - Has managed difficult situations in past, with support systems
 - Depends on others to manage difficult situations
 - Coping skills and ability to deal with stress includes patient's ability to:
 - Deal with loss or change
 - Manage loss of adaptive abilities
 - Concentrate and reduce anxiety, frustration, and irritability
 - Identify stressful situations
 - Identify illness progression, if indicated
 - Identify and access resources
 - Decision-making and negotiating skills include patient's ability to:
 - Understand when and what decisions need to be made
 - Make decisions alone
 - Obtain support to make decisions, if indicated
 - Be willing to make necessary changes to fit current situation
 - Plan to manage enforced or needed lifestyle changes includes patient's ability to:
 - Verbalize steps to take to fit current situation
 - Verbalize behavioral changes needed
 - Self-esteem or self-concept includes patient's ability to(2):
 - Make events turn out for the better
 - Express body image concerns
 - Express feelings of being powerless
 - Religion or spirituality; includes patient's attitudes regarding(2):

- Importance of religion or spirituality
- Being part of spiritual community
- Influence of faith on self-care
- **Mental and Emotional Status(15)(16)**
 - Appearance, grooming, and dress
 - Able to maintain personal hygiene with or without assistance
 - Able to manage clothing changes with or without assistance
 - Unable to manage clothing changes with available resources
 - Unable to manage personal hygiene
 - Socioeconomic factors impacting ability to perform hygiene or clothing changes (eg, unable to afford clothes)
 - Eye contact and facial expression
 - Able to make eye contact and nonverbal indicators of affect (eg, facial expression)
 - Unable to make eye contact and nonverbal indicators of affect (eg, facial expression)
 - Not applicable
 - Motor behavior
 - Restlessness, agitation, or lethargy
 - Spontaneous movement
 - None noted
 - Speech and communication
 - Speech clarity
 - Speech clear
 - Slurred or mumbled speech
 - Speech disabilities addressed with support devices (eg, white board, writing tablets)
 - No speech
 - Makes self understood
 - Understood - can express ideas and wants
 - Usually understood - may need prompting with words
 - Sometimes understood - ability to make concrete requests limited
 - Rarely or never understood
 - Ability to understand others
 - Understands
 - Usually understands - comprehends most conversation
 - Sometimes understands - responds appropriately to simple, direct communication only
 - Rarely or never understands
 - Thought processes
 - Thoughts coherent and relevant to conversation
 - Demonstration of loose associations, confabulation, or flight of ideas
 - Cognitive skills for daily decision making
 - Decisions consistent and reasonable
 - Some difficulty in new situations only
 - Decisions poor, cues or supervision required
 - Never or rarely made decisions
 - Orientation and level of awareness includes patient's ability to:
 - Report correct year
 - Missed by more than 5 years
 - Missed by 2 to 5 years
 - Missed by 1 year
 - Correct
 - Report correct month
 - Missed by more than 1 month
 - Missed by 6 days to 1 month
 - Accurate within 5 days
 - State day correctly
 - State time correctly
 - Identify self
 - Identify where they are (eg, home)
 - Identify current circumstance or situation
 - Memory deficit(17)
 - Problems recognizing familiar persons or places
 - Inability to recall events of past 24 hours, or significant memory loss
 - Short-term memory loss
 - Able to repeat 3 words immediately after given

- One try
 - Two tries
 - Three tries
 - Short-term memory: ability to recall 3 words given 5 minutes prior (check if yes)
 - Long-term memory: ability to respond to questions about past (check if yes)
 - Dementia screening
 - Mental status examination
- Depression screening (Check all that apply.)
 - Symptoms present(18):
 - Feeling down or hopeless and loss of interest in activities
 - Sleep pattern changes (eg, sleeps more or less, cannot fall or stay asleep)
 - Fatigue
 - Appetite changes (eg, overeating, loss of appetite)
 - Bad feelings about self (eg, being a failure, letting people down)
 - Concentration changes (eg, thoughts drift while reading or watching TV)
 - Movement or speech changes (eg, moving or speaking slowly, being fidgety)
 - Suicidal thoughts or actions
 - Easily annoyed
 - Duration of symptoms
 - 1 day or less
 - 2 to 6 days (a few days)
 - 7 to 11 days (patient has symptoms half the time during the period (eg, for 7 days, patient has symptoms 4 days out of the 7))
 - 12 days or more (almost every day)
- Anxiety(19)
 - Expresses feeling tense or worried most of the time
 - Can pinpoint times of day when anxiety occurs
- Psychosis(20)
 - Hallucinations
 - Delusions
- Behavioral symptoms
 - Physical behavioral symptoms directed toward others (eg, hitting, kicking, pushing, scratching, grabbing, abusing others sexually)
 - Verbally abusive (eg, threatening others, screaming at others, cursing at others)
 - Self injurious (eg, physical symptoms such as hitting or scratching self)
 - Socially inappropriate in public (eg, sexual acts, disrobing in public, throwing or smearing food or bodily wastes)
 - Disruptive (eg, verbal/vocal symptoms, such as screaming, disruptive sounds)
 - Resists care (eg, blood work, taking medications, ADL assistance)
 - Wandering
- **Social Support Systems and Home Environment(7)(8)**
 - Safety concerns present. See Home Safety Assessment  SR.
 - Transportability for follow-up care and to achieve ADL and IADL
 - Able to get out of home for needs (eg, keeping healthcare appointments or going to store) without assistance
 - Able to get out of home with assistance
 - Able to drive or needs another to provide means of transportation
 - Depends on others for transportation
 - Uses public transportation
 - Transportation support needed (eg, ambulance)
 - Caregiver situation and relationship(5)
 - Caregiver is family member.
 - Caregiver is from community (eg, neighbor, church).
 - Caregiver lives nearby.
 - Illness or condition has changed patient role in family or household and impacts care planning.
 - Patient willing and able to accept caregiver support.
 - Disagreements or conflict about patient's care occur.
 - Support system is available for intermittent assistance or emergencies, if needed.
 - Risk of social isolation is secondary to impairment in physical, cognitive, or behavioral abilities.
 - Patient or caregiver understands condition and medication.
 - Patient or caregiver understands information and has ability to manage crisis.
- **Financial Resources(8)(21)**
 - Able to manage financial implications of situation
 - Employment

- Able to maintain employment
- Able to return to work
- Unemployed
- Not applicable
- Caregiver financial support
- Government-assisted financial support (eg, Social Security)
- Knowledge of agencies and resources for financial support (eg, current financial aid, availability of financial aid, health insurance and coverage)
- Referred for financial support available for follow-up care (eg, other sources of income, medication programs)
- **Social Services(7)**
 - Using community resources available (eg, support groups, community organizations)
 - Patient and caregiver know how to access needed resources or agencies.
 - Able to access language interpreter for continued care, if indicated
- **Caregiver Assessment(4)(22)**
 - Caregiver able and willing to support patient's needs (eg, shopping, physical care, emotional support)
 - Caregiver responsibilities for patient
 - Primary caregiver
 - Hours or days per week spent giving care
 - Nature of responsibilities
 - Length of time providing care to date
 - Caregiver cultural background[A](10)
 - Citizenship
 - Ethnic heritage
 - Primary language
 - Other family responsibilities (eg, children to care for, expected role within own family)
 - Child care available, if indicated
 - Able to juggle day-to-day decisions and activities
 - Employment and impact of caregiving on employment
 - Interference of caregiving and ability to work
 - Financial impact with depletion of funds related to lack of employment or cost of healthcare
 - Caregiver support
 - Knowledge of respite care or relief from caregiving responsibilities
 - Knowledge of community resources (eg, American Heart Association, local adult day care, internet sites)
 - Caregiver stress, coping, and depression(5)
 - Living conditions changed
 - Conflict within family
 - Time and energy needed
 - Information or skills inadequate
 - Social isolation present
 - Depression symptoms present
 - Caregiver's health status and self-care (eg, meets own physical and emotional needs)
 - Perception of health status
 - Poor
 - Temporary health issues
 - Good
 - Excellent
 - Understands and obtains adequate time for eating, sleeping, and exercise
 - Care of personal chronic conditions
 - Requires support services to maintain health
 - Caregiver's health literacy; caregiver is able to:
 - Understand directions on appointment slips
 - Understand medical forms
 - Express confidence in:
 - Filling out medical forms
 - Following directions on medication bottles
 - Understand written information from healthcare provider about medical condition
 - Read alone or requires another to read hospital materials

Interventions

Psychosocial Interventions

- **Maintenance of Working Relationship(23)**

- Patient and caregiver understand roles and responsibilities of healthcare professionals.
 - Healthcare professionals are available to meet patient's needs.
- **Creation of Supportive Environment**
 - Verbal information
 - Presence of active advance directives (eg, living will, power of attorney for medical decisions)(24)
 - Desire of patient or caregiver to complete advance directives(2)
 - Discussion of advance care planning includes(25):
 - Deactivation of devices
 - CPR
 - Nutritional support
 - Mechanical ventilation
 - Hemodialysis
 - Chemotherapy or radiation therapy
 - Provide hope.
 - Praise and celebrate success.
 - Provide end-of-life support as needed.
 - Answer questions.
- **Educational Interventions(22)**
 - Assess and utilize patient and caregiver learning preferences and abilities.(2)
 - Education provided as:
 - Written information, such as books, handouts, pamphlets, or directions
 - Verbal information, including teaching about procedure or condition, educating with audiotapes or videotapes, and answering questions
 - Support groups or classes
 - Support coordinated with school system, if school-age child(26):
 - Managing schoolwork at home
 - Re-entry into school when cleared by physician
 - Impact from limitations on ability to participate in normal activities
 - Transportation arrangement to school as needed
 - Create a health management plan with school and doctor (eg, Individualized Education Program (IEP) or 504 plan).
- **Behavioral Strategies and Problem-Solving(15)**
 - Patient is able to employ behavioral strategies (may relate to diet changes, exercise, relaxation, rehearsal, and other active coping techniques).
 - Patient needs support to employ behavioral changes (eg, address barriers, further education).
 - Patient is able to employ problem-solving skills with support (ie, problem identification, solving process, modeling, and develop resources).
 - Patient is unable to employ problem-solving skills (ie, problem identification, solving process, modeling, and develop resources).
- **Cognitive Strategies(15)**
 - Patient has cognitive deficit that requires caregiver support.
 - Patient does not recognize distortions in own thinking or provides rationale for distorted thoughts.
 - Patient requires help to integrate new information.
 - Patient requires help to anticipate future needs.
- **Providing Referrals and Managing Healthcare Benefits(23)**
 - Patient needs assistance arranging appointments.
 - Patient needs assistance or reinforcement in understanding health status.
 - End-of-life referrals (eg, palliative care, hospice)
 - Community resources (eg, support groups)
- **Psychosocial Interventions**
 - Examples of psychosocial interventions patient may use include:
 - Guided imagery(14)
 - Stress management
 - Self-management programs(27)
 - Psychosocial support(2)
 - Trusting relationship established between clinician and patient or caregiver

Interventions for Caregivers

- **Depression(4)(5)**
 - Teach caregiver to recognize symptoms of depression in self and patient.
 - Explain treatment options, including counseling and medication, and arrange referrals, as appropriate.
 - Provide psychosocial support by allowing caregiver to express anger, frustration, anxiety, and other emotions.
 - Assess suicidal ideation, and provide intervention and counseling as needed.

- Identify and refer to community resources, such as financial counseling agencies and support groups.
- **Caregiver Burden(4)(5)**
 - Emphasize importance of maintaining social contacts and support, and asking for help, as appropriate. Social isolation is a particular risk factor for caregivers.
 - Evaluate relationship between caregiver and patient. Be alert for deterioration and negative feelings. Assess for signs of abuse or neglect in patient.
 - Assess how caregiver is prioritizing tasks and how much time is spent on their own needs. Encourage self-care.
 - Encourage caregiver to use their religious affiliation, including any help that may be offered from place of worship, along with use of faith and prayer.
 - Arrange for home health aides, transportation, respite, meals on wheels, and any other forms of physical support for patient and caregiver that are appropriate and affordable.
- **Physical and Emotional Health(5)**
 - Assess caregiver's ability to perform tasks of caregiving.
 - Assess physical and emotional status of caregiver. Encourage caregiver to seek healthcare for symptoms and conditions, as appropriate.
 - Address feelings of grief, guilt, powerlessness, and other emotions. Allow caregiver to discuss and express these feelings, as appropriate. Refer as needed.
 - Identify and refer to resources, such as financial and community information and support groups.

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6/25/24, 7:02 PMSR - Psychosocial Assessment

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Footnotes

[A] Cultural experiences define how care recipients and caregivers receive information and how they make choices.(10) [A in Context Link 1, 2]

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